



30/05/2026

Client-ready insight report
Dataset:- ↓ Samaritans Crisis Stats

Prepared For

NHS Mental Health Services – Client Insight Report

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DATA ANALYTICS INTERN

Reporting Period

Analysis based on official suicide statistics
through 2024

Executive Summary

This report presents an analysis of suicide trends in England and Wales using official national statistics. The objective is to identify long-term crisis trends, demographic risk patterns and regional variation that can support NHS mental health planning and crisis prevention strategies.

The analysis found that suicide deaths increased to 6,190 in 2024, with an age-standardised suicide rate of 11.4 per 100,000 population. Male suicide mortality remained substantially higher than female mortality, accounting for approximately three-quarters of all suicide deaths. Regional analysis identified significant geographical variation, with the North East recording the highest suicide rate and London recording the lowest.

These findings indicate the continued importance of targeted suicide prevention initiatives, improved crisis-support accessibility and region-specific intervention strategies.

Project Objective

The purpose of this analysis was to evaluate publicly available suicide statistics and identify patterns relevant to mental health crisis support and prevention planning.

The analysis sought to answer the following questions:

- How have suicide deaths changed over time?
- Which demographic groups are at greatest risk?
- Which regions experience the highest suicide burden?
- What implications do these findings have for NHS mental health services?

Data Sources

The analysis was based on official suicide statistics published for England and Wales.

Primary Data Source

Suicides in England and Wales Dataset (2024 Release)

Key tables used:

Table	Description
Table 1	Age-standardised suicide rates by sex (1981–2024)
Table 4	Age-standardised suicide rates by English region

Variables Analysed

- Total suicide deaths
- Age-standardised suicide rates
- Male suicide deaths and rates
- Female suicide deaths and rates
- Regional suicide rates
- Historical trend indicators

Methodology

Data was extracted from the official statistical workbook and cleaned using Python.

The analytical workflow included:

1. Data extraction and cleaning
2. Validation of numeric variables
3. Exploratory Data Analysis (EDA)
4. Trend analysis
5. Gender comparison analysis
6. Regional variation analysis

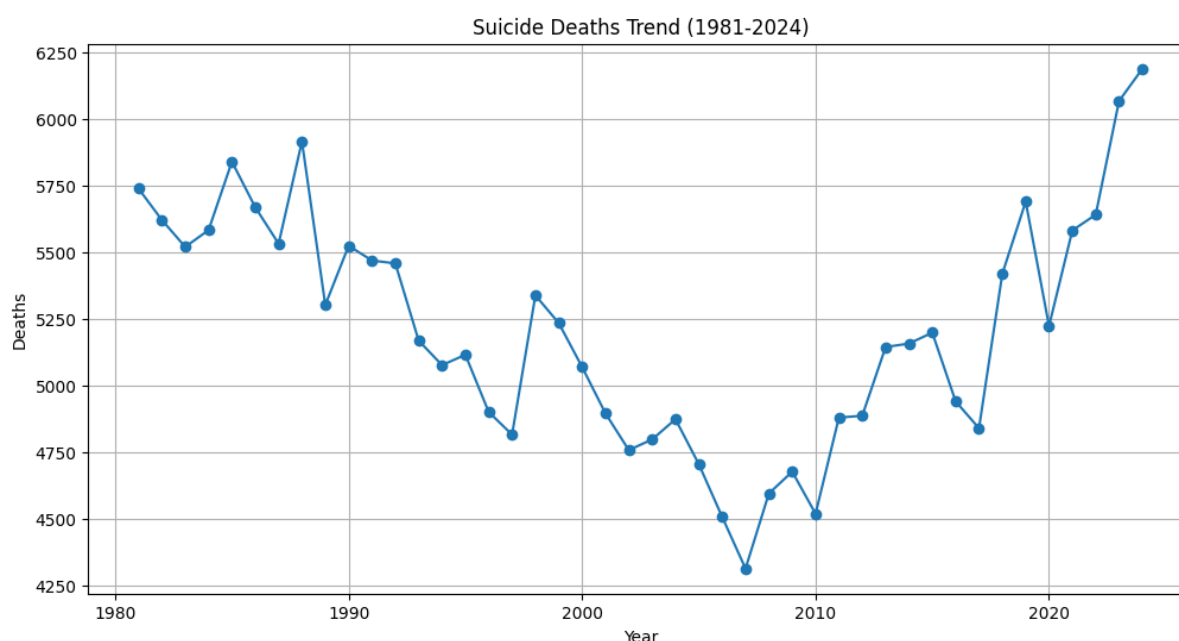
7. Insight generation and reporting

Visualisations were produced to support interpretation of trends and geographic differences.

Key Findings

Long-Term Suicide Trends

Analysis of historical data shows that suicide remains a persistent public health challenge.



Key Observations

- 6,190 suicide deaths were registered in 2024.
- The age-standardised suicide rate reached 11.4 deaths per 100,000 population.
- Recent years show a gradual increase compared with many earlier years in the series.
- Suicide mortality remains above levels observed in several previous decades.

NHS Implication

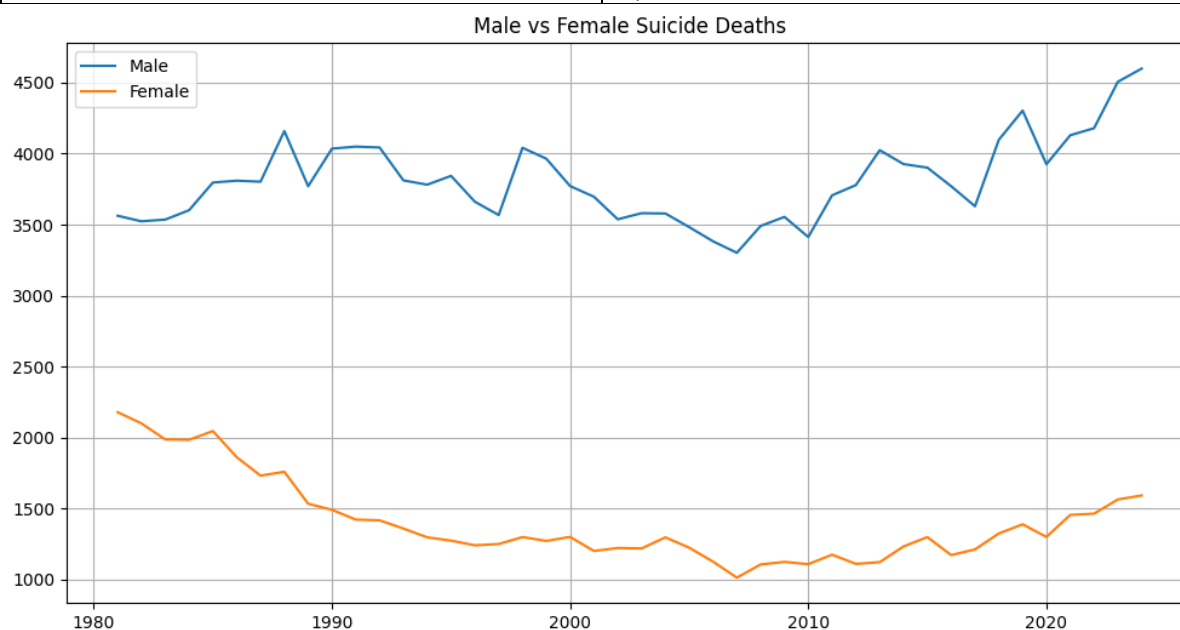
The increase in suicide deaths highlights continued demand for crisis intervention services, community mental health support and preventative outreach programmes.

Gender Analysis

A substantial gender disparity exists across the entire time series.

2024 Results

Gender	Deaths
Male	4,599
Female	1,591



Key Insights

- Male suicide deaths were almost three times higher than female deaths.
- Males consistently represent the highest-risk population group.
- The gender gap has remained evident throughout the historical data.

NHS Implication

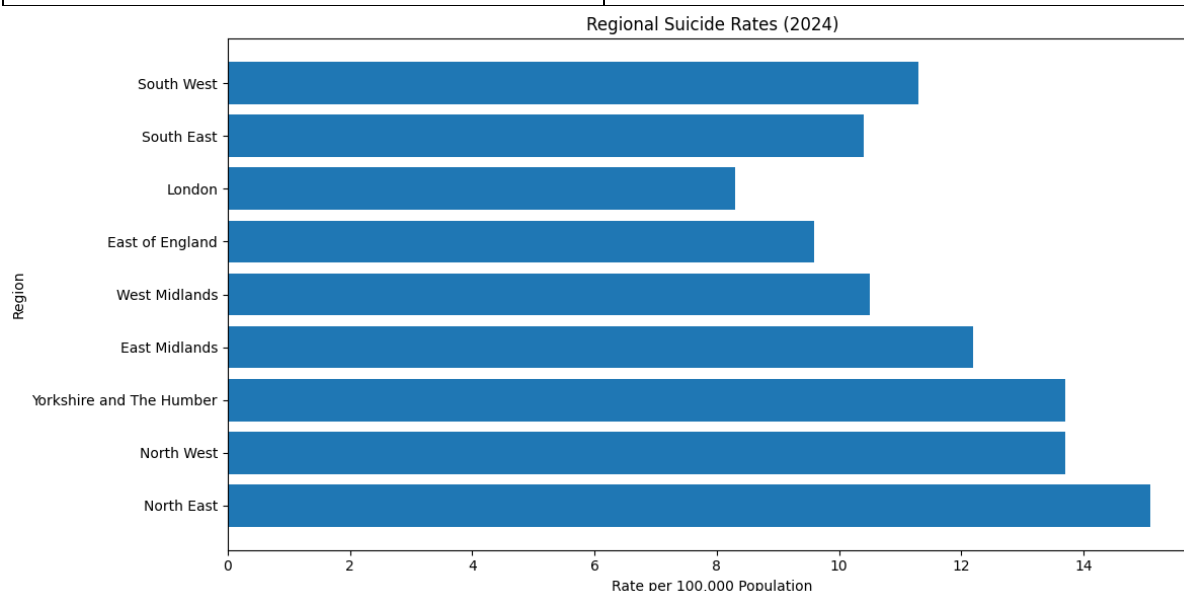
Mental health interventions should continue prioritising male engagement, early intervention programmes and targeted awareness campaigns.

Regional Variation

Significant differences were observed between English regions.

2024 Regional Suicide Rates

Region	Rate per 100,000
North East	15.1
North West	13.7
Yorkshire and The Humber	13.7
East Midlands	12.2
South West	11.3
West Midlands	10.5
South East	10.4
East of England	9.6
London	8.3



Key Insights

- North East recorded the highest suicide rate.
- London recorded the lowest suicide rate.
- Rates varied considerably across regions.

NHS Implication

A uniform national approach may not adequately address local needs. Regional targeting and resource allocation should be informed by local suicide burden.

Limitations

Several limitations should be considered when interpreting the findings.

Data Limitations

1. The dataset contains registered suicide deaths rather than real-time crisis contacts.
2. Monthly or seasonal crisis-call volume data was not available.
3. Individual-level socioeconomic variables were not included.
4. Findings represent aggregate population trends and should not be interpreted at an individual level.

Analytical Limitations

- Regional comparisons do not explain underlying causes.
- Additional factors such as deprivation, unemployment and service accessibility were not available within the dataset.

Recommendations

Based on the findings, the following recommendations are proposed.

Recommendation 1

Strengthen Male Suicide Prevention

Expand targeted outreach programmes focusing on men, particularly working-age populations.

Recommendation 2

Increase Regional Resource Targeting

Prioritise additional support capacity within higher-risk regions including:

- North East
- North West
- Yorkshire and The Humber

Recommendation 3

Enhance Early Intervention Services

Invest in earlier identification of mental health risks through:

- Primary care
- Community services
- Crisis helplines

Recommendation 4

Improve Data Integration

Combine suicide statistics with:

- NHS mental health service data
- Crisis helpline activity
- Community mental health indicators

to provide a more comprehensive picture of population mental health risk.

Recommendation 5

Expand Monitoring and Reporting

Develop interactive dashboards to support ongoing monitoring of:

- Suicide rates
- Regional trends
- Demographic risk groups

Conclusion

The analysis demonstrates that suicide remains a significant public health concern across England and Wales. The 2024 data indicates rising mortality levels, persistent gender disparities and notable regional variation.

The findings support continued investment in crisis prevention, targeted intervention programmes and data-driven mental health planning. By focusing resources on high-risk populations and regions, NHS organisations can strengthen prevention efforts and improve outcomes for vulnerable individuals.